

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

2

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°015/MVB_29/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (22)	ITURI : Aru, Aungba, Bambu, Damas, Bunia, Gety, Kilo, Komanda, Lita, Mangala, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma, Oicha, Kalunguta, Katwa, Kyondo and Beni SOUTH KIVU : Miti-Murhesa
Reporting date	May 29, 2026
Publication date	May 30, 2026


 Total
confirmed cases

 Total deaths
among
confirmed cases

 Cumulative Cases
Suspects

 Occupancy rate
among suspects

 Occupancy rate
among experienced staff


Healed


 Cumulative
confirmed cases of CTE

0. HIGHLIGHTS

- Three (3) vehicles out of the fifteen (15) expected have been received; 1,800 motorcycles from the central government are also expected to strengthen response operations.
- Guided tour organized with the National Director of the PNHF at Bunia airport in order to improve the monitoring and control circuit.
- Notification of fifty-four (54) new confirmed cases as of May 29, 2026.
- Geographic expansion of the epidemic with six (6) new health zones affected in the Ituri province.

I. CONTEXT

The Ituri province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction sites that attract people from other territories within the province and from neighboring provinces.

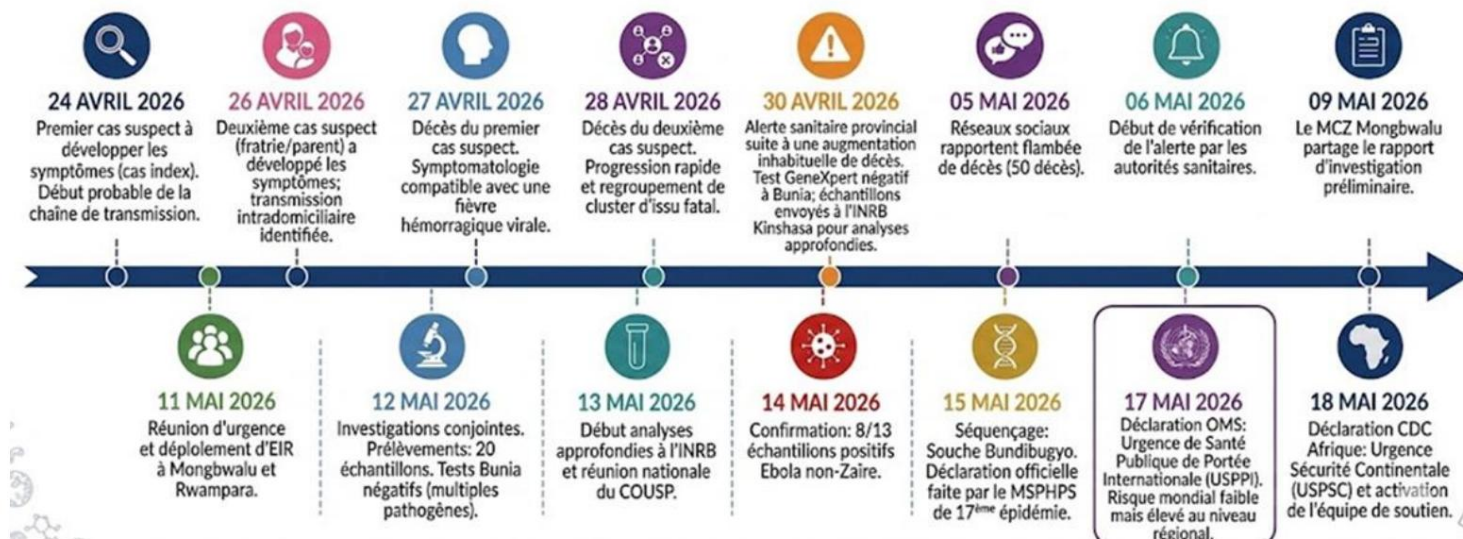
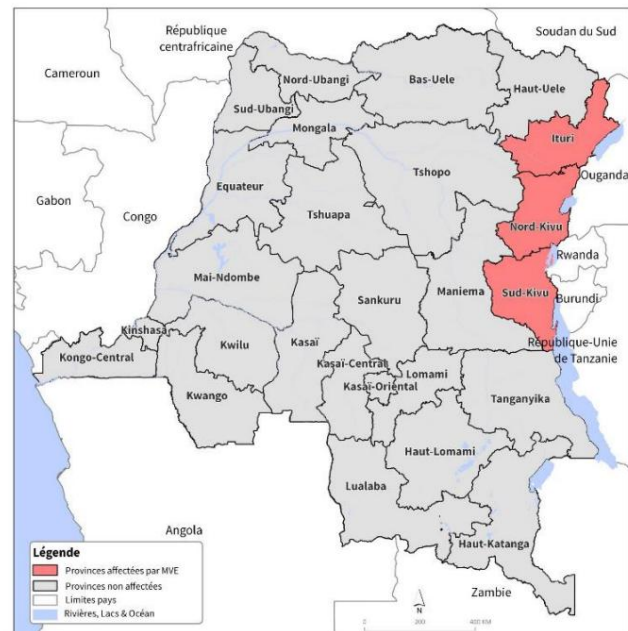
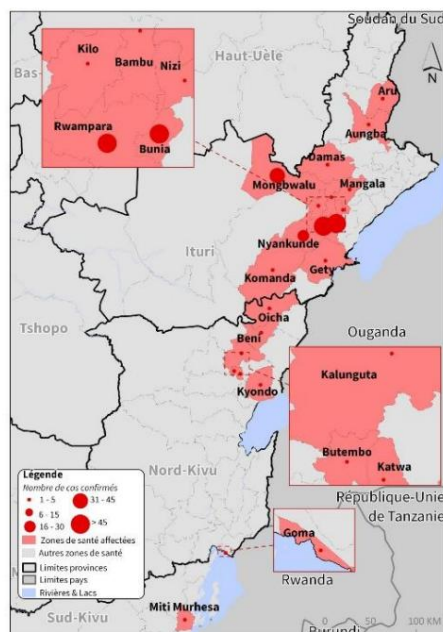


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of May 29, 2026.

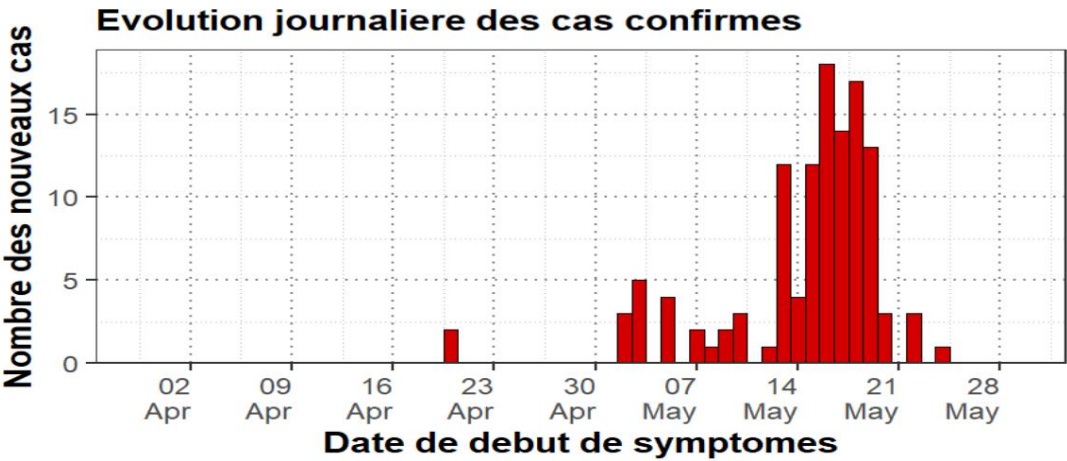
II. EPIDEMIOLOGICAL SITUATION UPDATE

- Fifty-four (54) new confirmed cases were notified during the day of May 29, 2026.
- As of May 29, 2026, the cumulative number of confirmed cases stands at 263, including 245 in Ituri, 15 in North Kivu and 3 in South Kivu.
- Three provinces are currently affected by the epidemic: Ituri, North Kivu and South Kivu.
- 6 new health zones affected as of May 29, 2026. The number of health zones affected remains at 14 in Ituri and at 22 nationally.
- The Ituri province remains the epicenter of the epidemic with 14 health zones affected out of 36: Aru, Aungba, Bambu, Damas, Bunia, Gety, Kilo, Komanda, Lita, Mangala, Mongwalu, Nizi, Nyankunde and Rwampara.

- In North Kivu, 7 out of 35 health zones are affected: Goma, Beni, Butembo, Oicha, Kalunguta, Katwa and Kyondo.

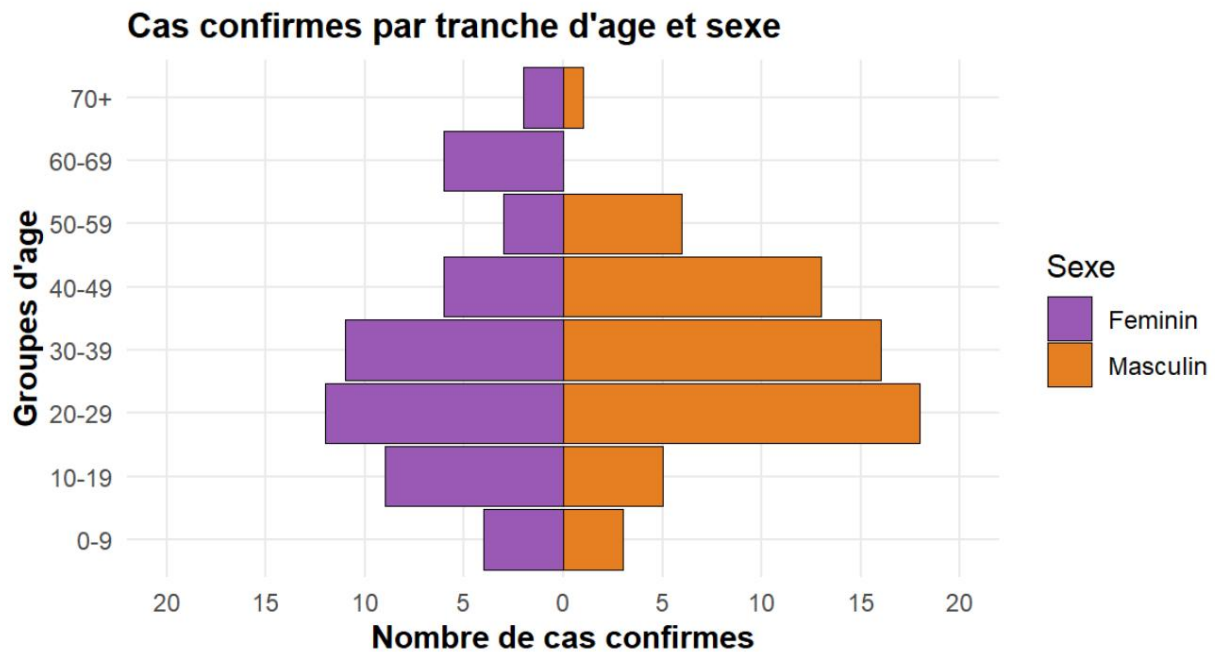
Table I. Key indicators of epidemiological surveillance in the three provinces as of May 29, 2026

Provinces	Health zones	New case suspects	Number of suspicious deaths	New case confirmed	Accumulation of case Confirmed	Cumulative number of confirmed deaths	Healed	Number of contacts	Follow-up contacts (<21 days)	Rate of Follow-ups
Ituri	Aru	-	-	0	2	1	-	107	0	0.00%
	Bamboo	8	-	0	2	2	-	323	101	31.27%
	Bunia	19	19	21	80	8	-	476	0	0.00%
	Kilo	-	-	0	2	-	-	41	0	0.00%
	Mongbwalu	7	0	8	47	10	-	235	0	0.00%
	Nizi	-	-	0	2	-	-	288	40	13.89%
	Nyankunde	7	1	2	19	1	-	435	346	79.54%
	Rwampara	13	2	12	65	13	-	766	0	0.00%
	Unidentified zone	-	-	0	6	-	-	-	-	-
	Mangala	-	-	0	1	-	-	-	-	-
	Damascus	1	0	0	1	-	-	-	-	-
	Aungba	-	-	0	1	-	-	-	-	-
	Gety	3	0	-	1	-	-	-	-	-
	Komanda	-	-	-	1	-	-	-	-	-
	Jiba	2	0	-	-	-	-	-	-	-
	Lita	-	-	-	-	-	-	-	-	-
	No record	-	-	1	1	-	-	-	-	-
	Ituri subtotal	60	13	8 52	14	35	0	2671	487	18.23%
North Kivu	Butembo	-	-	-	245 5	2	-	24	20	83.33%
	Goma	-	-	-	1	-	2	66	66	100.00%
	Katwa	-	-	-	4	3	-	112	110	98.21%
	Karisimbi	-	-	-	0	-	-	249	232	93.17%
	Kyondo	-	-	-	1	0	-	16	-	-
	Oicha	-	-	-	1	-	-	32	32	100.00%
	Kalunguta	-	-	-	1	1	-	20	20	100.00%
	Manguripa	-	-	-	0	0	-	10	-	-
	Blessed	-	-	-	2	-	-	-	-	-
	North Kivu subtotal	0	0	0	15	6	2	529	480	90.74%
South Kivu	Miti Murhesa	-	-	2	3	1	-	ND	-	-
	South Kivu subtotal	0	0	2	3	1	0	0	0	-
Total		60	13	54	263	42	2	3200	967	30.22%



Riposte MVE, mai 2026. Donnees incluses dans l'analyse : n cas confirmes = 120.

After a few sporadic cases observed in late April and early May, a gradual and then rapid increase in confirmed cases of Ebola virus disease (EVD) was noted during May 2026, with a peak observed around May 17–18. A slight decrease in reports was observed at the end of the period, although transmission remained active.



acker - Riposte MVE, mai 2026. Données incluses dans l'analyse : n cas confirmés = 115.

The 20-49 age group is the most affected, with a predominance of males. The extreme age groups (0-9 years and ≥70 years) remain the least affected.

III. RESPONSE ACTIONS NSE

IV.1. COORDINATION

- Continuation of the mission of the Minister of Health-INSP •
- Field missions in the health zones of the city of Bunia with the Director General of INSP.

IV.2. EPIDEMIOLOGICAL SURVEILLANCE

- Monitoring pillar meeting regarding the pillar's reorganization, with integration of epidemiologists from Kinshasa in the sub-pillars and health hotspot zones.
- Supporting teams in the affected health zones.
- Development of case narratives and follow-up of contacts in the Nizi health zone.
- Supervision of Ebola surveillance activities in the Rwampara health zone, harmonization data and coaching on alert management in the Bunia health zone.
- Implementation of routine field monitoring activities: reporting of alerts, investigations, active case finding, contact listing and follow-up, retrospective documentary review of cases and deaths, etc.
- For May 29, 2026, 132 alerts were reported, of which 85 (64.3%) were investigated and 71 (83.5%) validated, of which 54 (76.0%) were subject to sampling.



Point of Entry and Point of Control (PoE/PoC) Surveillance:

- Development of a schedule of intensive activities to be carried out over twelve (12) days at the following points input (PoE).
- Joint visit of the PoE and UMIR teams to the airport for the evaluation of the military terminal and the organization of health control activities.
- Data encoding and updating of the PoE/PoC database.
- Updating in progress of the mapping of population movements in the health zones of Bunia and Rwampara.

Table III. Summary of PoE/PoC data as of 29/05/2026

Indicators at PoE/PoC	TOTAL
Travelers who have used the PoE/PoC	18,239
Screened travelers	18,010 (98.7%)
Travelers who have washed their hands	16,936 (92.8%)
Travelers made aware of Ebola virus disease	17,968 (98.5%)
Listed contacts	0
Alerts raised	0

IV.3. LABORATORY

- Continued analysis at the Bunia laboratory and the INRB Kinshasa.

Table IV. Update on the laboratory situation as of 29/05/2026

Key indicators	29/05/2026	Total
Samples received	70	758*
Re-sampling (control)	0	8, of which 6 are negative
Number of samples analyzed	70	ND
Number of positive cases	54	220
Positivity rate	77.1%	N / A

**This data is only for the Bunia laboratory*

IV.4. INFECTION PREVENTION & CONTROL

- Continued training of trainers and support partners on the complete PCI package (J4).
- Decontamination of a household of a confirmed case in the Ngezi district as well as four (4) rooms of the hospital UNIBU (AS Ngezi).

- Continued briefing of forty (40) healthcare providers on the introduction of IPC and precautions standards and complementary (AS Rwambuzi and CH Rwankole).

1. ZS Bunia

- Four (4) EDS carried out out of six (6) alerts: 1 at CH Rwankole, 2 at HGR Bunia, 1 at Top Santé and 1 secured at HGR Bunia.
- Donation of ten (10) pairs of boots to the PCI commission.

2. ZS Rwampara

- Briefing of four (4) health providers and one (1) hygienist on the importance of hand washing, the preparation of 0.05% chlorinated water and the installation of a temporary triage at the entrance of the structure.
- Four (4) EDS carried out out of five (5) alerts: 2 at AS Rwampara, 1 at CME and 1 at AS Shari.
- Two (2) decontamination operations carried out at AS Rwampara.

3. ZS Bambu

- Briefing of fourteen (14) hygienists on the general concepts of Ebola virus disease and decontamination of the isolation room of a confirmed case at Bambu General Referral Hospital.

IV.5. HOLISTIC CARE

- First negative control obtained in seven (7) positive patients, awaiting their second control.
- Eleven (11) suspected cases escaped from Bambu General Referral Hospital due to lack of nutritional support.
- One (1) critical case taken care of at the Bunia General Referral Hospital.
- Continued patient care.
- Continued active searching and sampling of discharged patients without results available.
- Monitoring of construction work at the Bunia CTE, CME and Rwampara.

Psychosocial interventions and mental health

- Seventy-four (74) results (reports), including sixteen (16) positive results from Nyankunde, Rwampara, Mongbwalu, Kilo, Mangala, Boga, Lita, Bambu, Nizi, Tchomia and Bunia, as well as four (4) deaths at the CTE of the HGR Bunia.
- Psychosocial support for four (4) persons deprived of liberty (PPL) in isolation in Rwampara and Nyankunde, as well as for four (4) families of persons who died in the context of the EDS at the HGR Bunia.
- Initial psychological support provided to twenty-six (26) new admissions and their contacts at Rwampara, Bambu, Nyankunde, Mongbwalu and Bunia.
- Briefing of clinical psychologists on the consultation form in Bunia and Rwampara.
- Support to EDS teams for obtaining consent for SWAB sampling in Rwampara and Bunia.
- Administration of self-assessment questionnaires (EDAS-21) to staff at CME, HGR Rwampara and Bambu.

Table V. Patient movement in healthcare facilities as of 29/05/2026

ISSUES		HGR	HGR	CME	HGR	HGR	HGR	HGR	Total
Report (Number of patients present in bed at the end of the previous day)		18	9	8	12	38	12	2	99
News	PPL	0	1	1	0	0	0	0	2
Today's admissions	Others	2	4		4	0	4	0	15
Outings	Healed	0	0	1	0	0	0	0	0
	Deceased	2			2	0	0	0	6
	No Case	0	1	0	0	0	0	0	0
	Escapees		0	1	3	0	11	0	15
	Transferred to HGR	1	0	0	0	0	0	0	0
Patients in isolation	Total	0	0	0	11	38	5	2	95
	NC confirmed			0	0	0		0	2
	AC confirmed			9	0	9	1		11
	Suspects	17	13	0	11	29	0		82
	Suspect Child < 5 years old	1 0 16 2	0 1 12 0	0 9 0	0	0	4 0	1 1 0	2

S: Suspects C: Confirmed ND: Not available

IV.6. CREC

A veteran FARDC soldier addressed the rumor about organ amputations by the teams of EDS 5 (ZS Mongbwalu).

- 21 journalists from local media in Bunia (Radios, TV and online media) have committed themselves.

ZS BUNIA:

- Awareness-raising at the FARDC General Staff for officers and soldiers of the 32nd Military Region on proper handwashing, adherence to the EDS (Emergency Health Diagnosis) and the registration of those wearing the signs at the CS (Health Center)

ZS RWAMPARA •

883 students and teachers from the MUSANGU, IBARA and MAKI MBUDHA primary schools, including 456 girls, were sensitized on preventive measures for Ebola virus disease (EVD) with a class-by-class strategy.

ZS Nizi

- Patients and caregivers in AS Lopa
- 30 Community liaisons, including 2 men, briefed on organizing home visits with feedback disease alerts, including Ebola virus disease (EVD), in the Lopa health center

ZS Bamboo

- 24 pregnant women sensitized on the prevention of Ebola during ANC in AS MANGAMBO.

ZS Aru

- Start of the CREC situational analysis in the health zone to identify gaps and priorities for the CREC.
- Quick briefing of community relays before their deployment for awareness-raising.

- Support for 2 strategic meetings with the Territory Administrator, then with the MCZ on the situation of the Ebola outbreak.

Key feedback and rumors gathered today

- Healthcare workers inject drugs into people showing signs of Ebola; response workers inflate Ebola case numbers to make money; there is only one trustworthy healthcare provider at the hospital, but there are fears he will be killed soon because he is not working with the response workers; response workers tell us there is no treatment for Bundibugyo Ebola, so how was the person who recovered cured?; when will the response end?; chlorinated water contains the Ebola virus (PoC Muzi Bala).

IV.7. LOGISTICS

- Delivery of IPC inputs to ESS in the Rwampara health zone, Bunia CME and Bunia schools (EP MAKI, EP NYANGABO and EP MUSANGO) with the support of WHO and OCHA.
 - Receiving inputs and equipment (Tyvek, US gown, gloves, tents) with support from UNICEF.
 - Reception of medicines provided by the UG-PDSS for the Bunia and Rwampara health zones.
- 27 **vehicles and 4 ambulances** were deployed to support the mobility of the pillars towards the axes Rwampara, Nyakunde, Bunia and Nizi.
- The two ambulances announced by MONUSCO have been made available for logistical support

IV.8. SECURITY

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm.

IV. CHALLENGES

- Reluctance towards post-mortem sampling: Formal refusal by families to have diagnostic tests (swabs) carried out on the bodies of the deceased in morgues.
- Absence of standardized Ebola Transit Centre and Treatment Centre in Ituri.
- Weakness in Contact Follow-up and Lack of Training for Stakeholders
- Persistence of gaps in the identification and rigorous monitoring of contact cases at the level of active and high-risk health zones.
- Shortages in isolation facilities: Total absence of essential medications at the Center Ebola treatment (ETC) and at the isolation block in Rwampara and Nyankunde.
- Supply failure of reference hospitals: Lack of supply of Essential Generic Medicines (EGMs) within the four targeted General Reference Hospitals (GRHs), weakening the overall care of patients.
- Lack of IPC equipment (PPE, DLM, chlorine) in North Kivu
- Low reporting of alerts by zones in the 3 affected provinces (Ituri, North Kivu and South Kivu) Kivu);
- Insufficient financial resources to support all pillars of the response in the health zones affected;
- Difficulty in transporting samples to laboratories in Goma and Kinshasa.
- Spread of rumors and proliferation of traditional treatment recipes.
- Lack of communication materials and tools.

V. PERSPECTIVES

- Training of service providers on the use of PCR RADIONE.
- Training of service providers on Ebola virus disease (EVD) and squamous cell carcinoma (SCC) surveillance in affected and high-risk health zones risk of three provinces being affected.
- Training of community health workers on Ebola virus disease and SBC surveillance in affected health zones and high risk in the province of ITURI.
- Finalize the development of the research protocol and the data collection tools for perception communities on Ebola, the response and actors in health zones affected by the Ebola epidemic.



Reception of 3 vehicles



Implementation of an EDS in the Bunia health zone.

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